Women's Healthcare Associates LLC

Prenatal Intakes Training Manual

Last Reviewed: 1/2024, Last Updated: 11/2022, 5/17/24 MN, 10/16/25 MN, 6/2026 MN

Women's Healthcare Associates LLC

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Intake Checklist

☐ Create pregnancy episode

- Naming convention is 'WHA Traditional Pregnancy [insert year the pregnancy started]'.

☐ Select 'Traditional' as prenatal model of care.

☐ Update patient's vitals with height and current weight.

☐ Add patient's pre-pregnancy weight to prenatal vitals and notes.

☐ Update dating section with patient's LMP/method being used to establish dating.

- Make sure to select the radio button next to the EDD you will be using.
- If the patient reports an LMP that is different than the LMP in the OB Intake phone note, send a high priority staff message to the site OB coordinator pool.

☐ Update Pregnancy Overview & Plan with planned delivery location and the patient's decision regarding blood products.

☐ Review allergies.

☐ Review medications and verify pharmacy.

☐ Thoroughly review & update all patient histories.

- Always refresh Care Everywhere first.
- OB Hx (year, delivery, complications, etc.), Medical Hx, Surgical Hx, Family Hx, Substance use, Sexual Hx and Social Hx.

☐ Review and fill out genetic screening section.

- Be sure to include any pertinent history into your Note template.

☐ Go through WBA with the patient if not completed online prior to intake.

☐ Confirm that patient has watched the prenatal education video. If not, encourage them to do so prior to their next provider visit.

☐ Review prenatal intake talking points. See section on Prenatal Intake Talking Points.

- Make sure to review that we are a male/female provider practice and the types of providers that they might see during their care.

 Wrap your visit with the patient by reminding them of the below:

- They will need to have a genetics decision by the next telehealth visit.
- Review of content going to their MyHealth account as well as resources on our website that can be accessed via the hyperlink in their AVS e.g. form on checking insurance for coverage of genetic tests

 Utilize the smart set in the plan activity to pull in the Progress Note, LOS, Pregnancy Diagnosis as well as placing nursing referral as appropriate.

 Sign the smart set under the provider responsible for the OB Workup.

 Pull pregnancy problem from the visit diagnosis section to the problems list and make generic by removing the gestational age/trimester.

 F2 through the note template to answer all required questions.

 Enter follow-up appointments in the wrap-up activity

 Enter external records; including imaging and blood work.

- This should be done either before the PI appointment if the records are available, or added as an addendum to the PI document once it's been signed when records are available.
- Records should be found in the Media tab in the patient's Chart Review activity.
    - If patient is transferring from another location that uses Epic, you will be able to see the labs in the "Lab" tab, but you will still need to enter them in the external lab results section so that they are visible when our providers do an H&P.

Prenatal Intakes via Phone Workflow

Workflow Contact: Training Team

Reminders: The schedule for this specific visit type can now be found under the TN Resource department (TN INITIAL PRENATAL APPT A. B, C and D). Furthermore, please note that lab work will be drawn during the 12-week OB workup appointment.

Steps to completing appointment via telephone:

1. Contact patient via Phone call.
2. When patient answers, introduce yourself and ask them to confirm their first/last name and DOB.

Epic prenatal intake documentation workflow:

1. From your schedule, single-click on the patient appointment.
2. Single -click **Check In** activity located on the top of the toolbar.

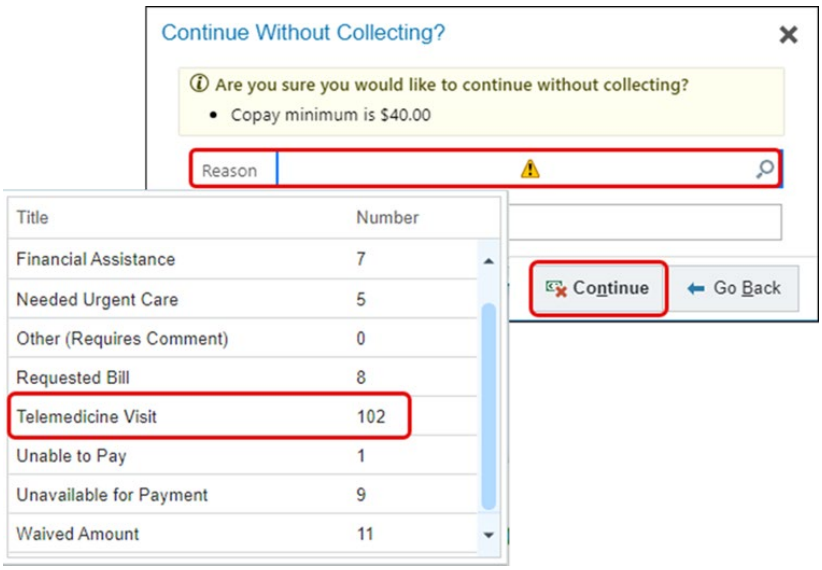


3. Once the **Registration** activity opens, click **Check in** 

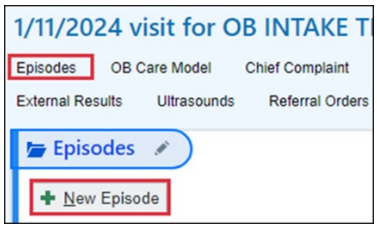
located on the lower right corner of your screen.
4. Ignore the warnings and click **Continue Anyway.**



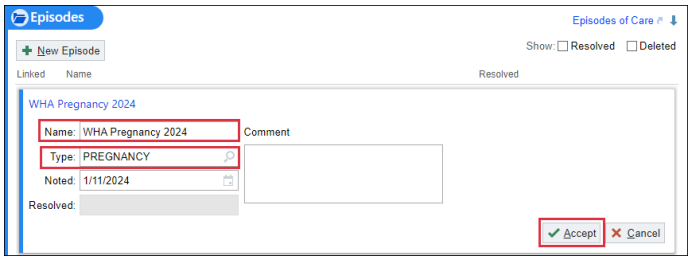
5. If the patient has a balance due, **the Payment Collection** activity opens.
6. To bypass this, click **Check In** button a second time.
7. Enter **Reason**: Telemedicine Visit.
8. Click **Continue.**



9. Click **Continue Anyway.**
10. You will not need visit labels, **click Cancel.**
11. Create the **Pregnancy Episode**.
    a. Click the **Episodes** activity from the navigation pane to open the section.
    b. Click the **New Episode** button.



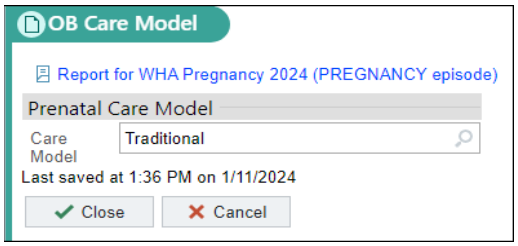
    c. Enter the **Name** of episode. Appropriate naming convention is **WHA Pregnancy 2024**. **Note*: *use the year that the pregnancy started.**
    d. Type: **Pregnancy**.
    e. Click **Accept**.



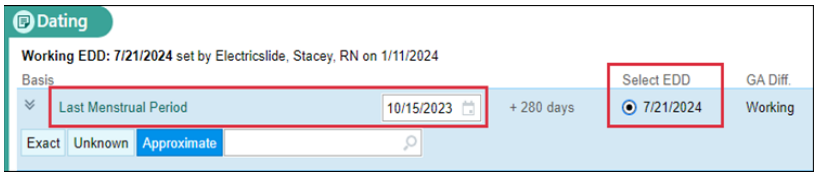
- It is important that the episode is created when the visit has been started so that the prenatal intake appointment is linked to the episode.

OB Care Model and Dating:

12. Select the **OB Care Model**.
    a. **Traditional** for all patients.



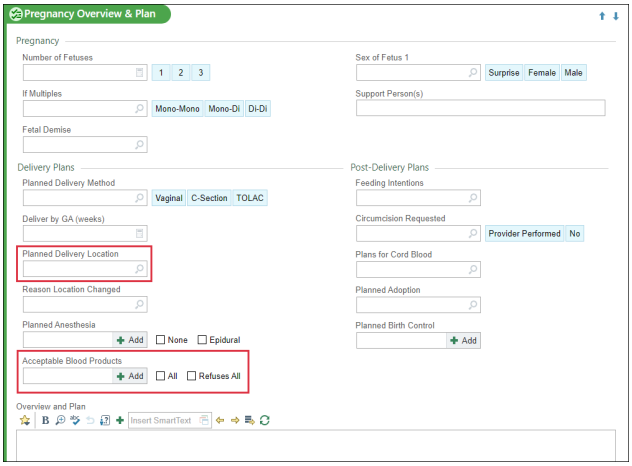
13. In the **Dating** section indicate dating if not already done (use LMP unless dating has been established via other means like ultrasound).
    a. Make sure to select the EDD by clicking the circular button next to the date so that the EDD will be calculated and displayed in the storyboard.
    b. If the patient reports an LMP that is different than the LMP in the OB Intake phone note, send a high priority staff message to the site OB coordinator pool.



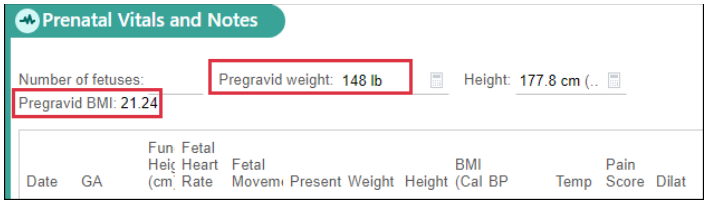
    c. If patient underwent IVF, click the link for further documentation instructions.

Vitals, Pregnancy Overview & Plan, Pregnancy Checklist:

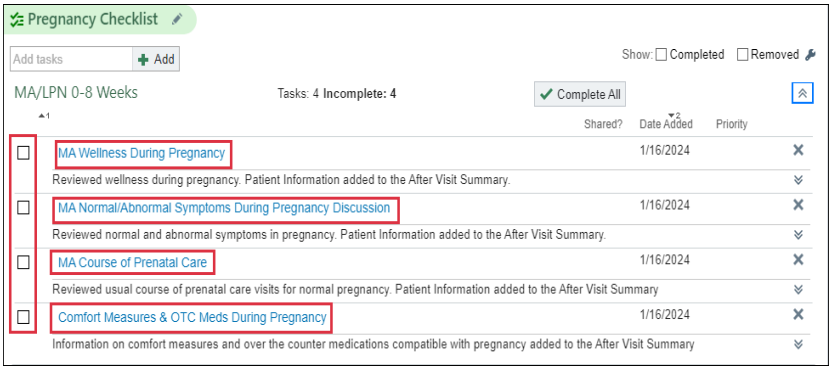
14. Add **Vitals**: ask patient to indicate **height** and **weight** (to calculate BMI)
    a. Be sure to add the unit of measurement for both; e.g. 150lbs, 65".
15. In the **Pregnancy Overview & Plan** fill in the **Planned Delivery Location** and **Acceptable Blood Products**, at least.



16. In the **Prenatal Vitals and Notes** section add in the **pre-pregnancy weight**, this should be the closest estimate to what they weighed on the date of their LMP.
    a. This will ensure the BMI can be added to all future progress notes during the pregnancy and allow providers to track weight gain throughout the pregnancy.

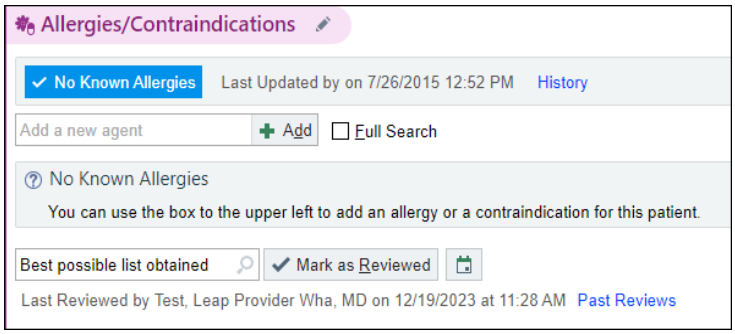


17. Pregnancy checklist: the **MA/LPN 0-8 weeks** section is completed by the support person doing the Prenatal Intake before 8-10 weeks.
18. Using the chevron  , **open the MA/LPN 0-8 Weeks**.
19. Check the **boxes** to add to the AVS.

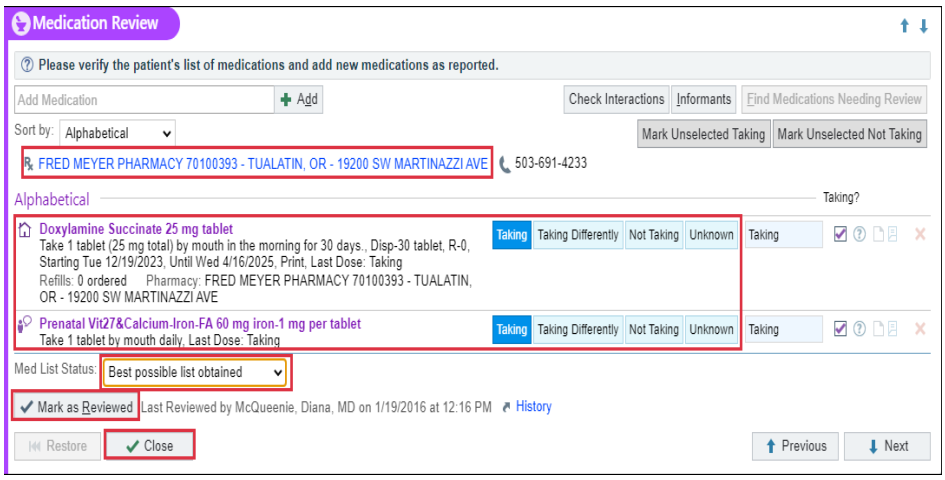


Allergies and Medications:

20. Review **Allergies** with the patient
    a. Allergies to medications, severe allergies, and other things the patient may come into contact with in the clinic should be listed here. If the patient has other, less severe allergies to certain foods like kiwi or to animal dander's, these should be listed in the medical history section, not the allergies section.

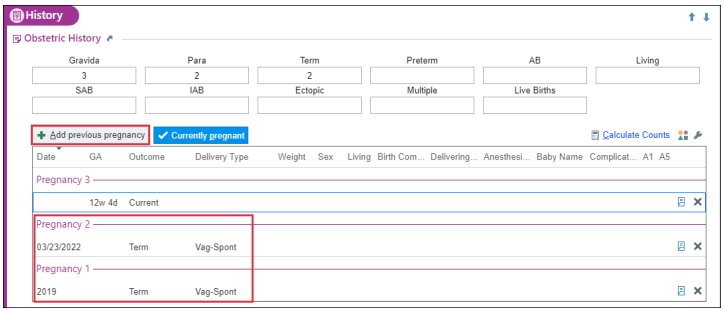


21. Click **Mark as Reviewed** once done reviewing.
22. In the **Medications** section, check the patient's pharmacy preference.
23. Review **Medications**, one by one, with the patient. Making sure the correct drug, dose, route, and frequency are listed, as well as any pertinent comments about how the patient takes the medication if it varies.
    a. Select **Best Possible List Obtained** then click the **Mark as Reviewed** button and **Close** the section.

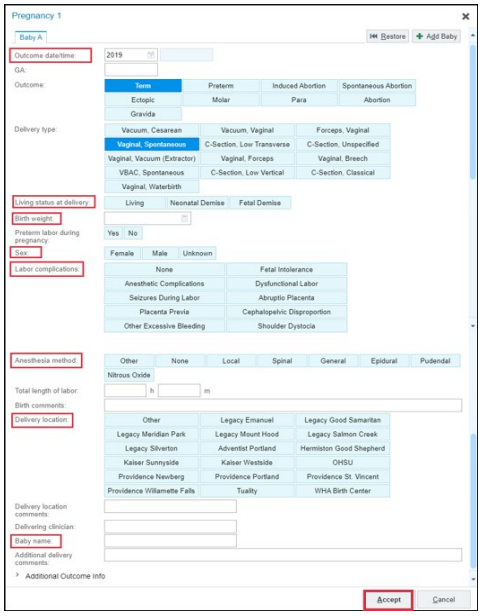


Histories:

24. Review **Histories**
    a. **OB History** – ensure the information in this section is correct by verifying it with the patient.

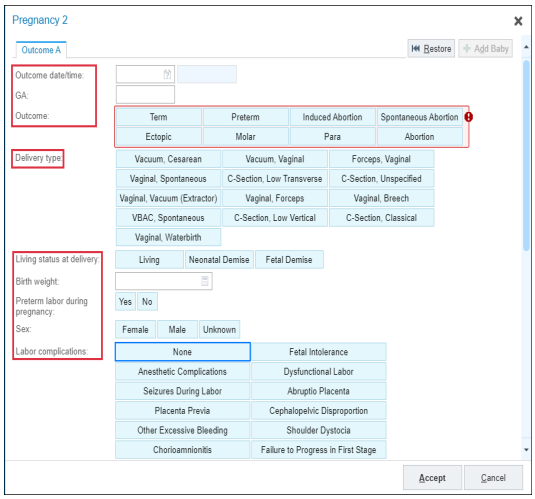


- If there are current pregnancies listed and you find information is missing, click on each pregnancy to review with the patient and update the fields.



- Add pregnancies by clicking **Add Previous Pregnancy** and filling in as much of the information as you can with the patient.
    - Delivery date
    - Delivery type
    - Complications

- Whether the child is living or not.



- Once verification is done, click **Calculate** 

and Epic will fill in the GP boxes on its own.

b. **Medical History**
   - If the patient does not have any pertinent medical history, search for **no known health problems**
   - Change each diagnosis from **Provider approved** to **from patient questionnaire**
   - Ask the patient if she has a history of chicken pox or of getting the vaccine and document which in the comment box.

c. **Surgical History**
   - If the patient has no prior surgeries, search for **other surgical history**, add the comment of **no prior significant procedures**, then select **from patient questionnaire.**

d. **Family Medical History**

e. **Sexual Activity**

f. **Alcohol Use**

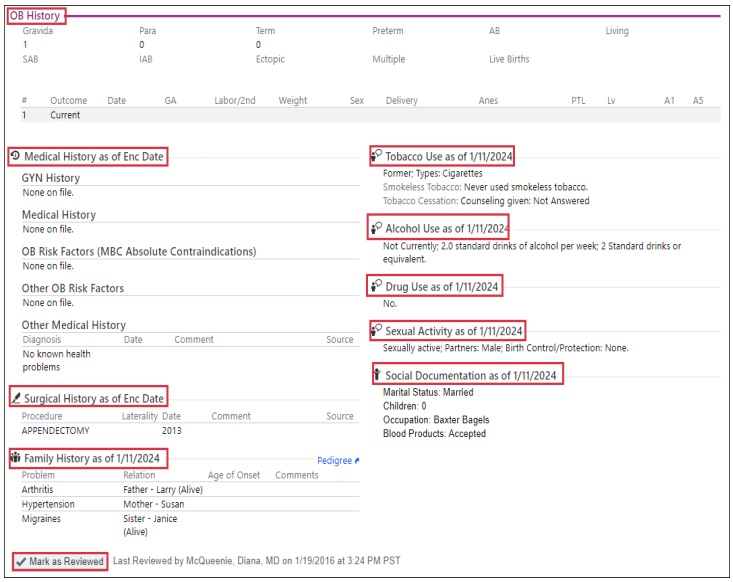
g. **Tobacco Use**

h. **Vaping**

i. **Sexual History**
   - Make sure to ask and fill in the answers to all of these questions. It will help the provider to gauge whether the patient needs additional screening or not.

j. **Social History**

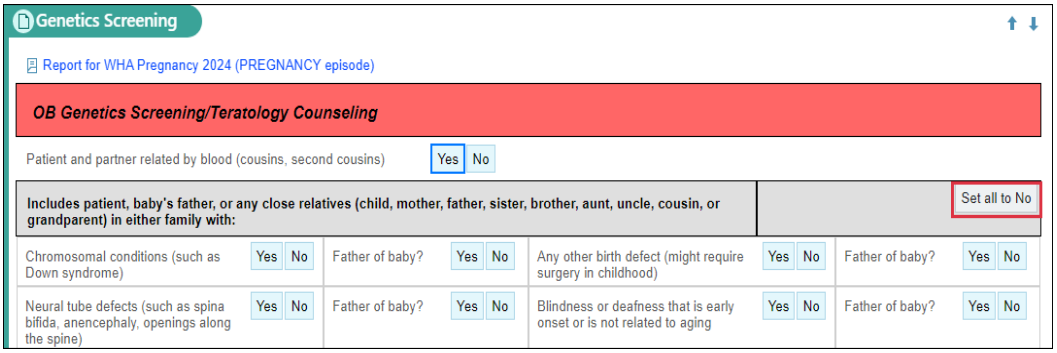
k. **Social Documentation**
- Marriage status
- Children
- Occupation
- Religious Preferences
- Blood Products and whether the patient accepts them or not



25. **Mark as Reviewed** once the histories section is completed then click **Close**.

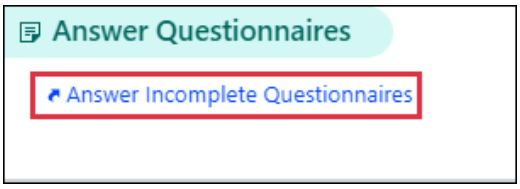
Genetic Screening:

26. In the **Genetics Screening** section, it is most efficient to click **Set All to No**, then change what applies to the patient to **Yes**.
Note: Be sure to include any pertinent history into your Note template.

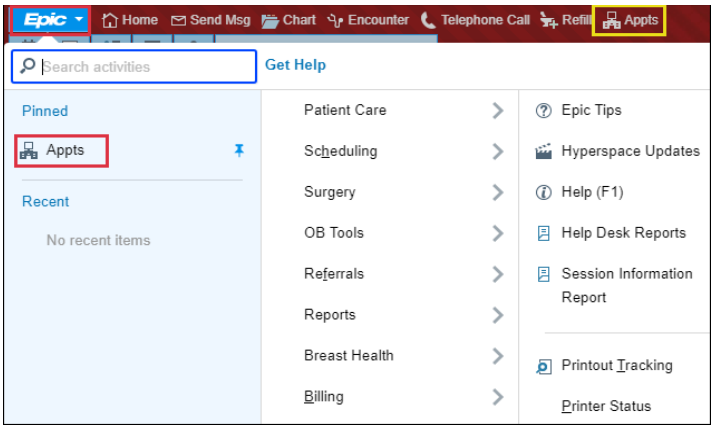


Complete the **Well Being Assessment** with the patient. Skip to step 23 if already completed online.

27. Go to the **Screening** tab and click the hyperlink to go through the Well Being Assessment with the patient.
28. Click on the **Answer Incomplete Questionnaires** hyperlink to open.



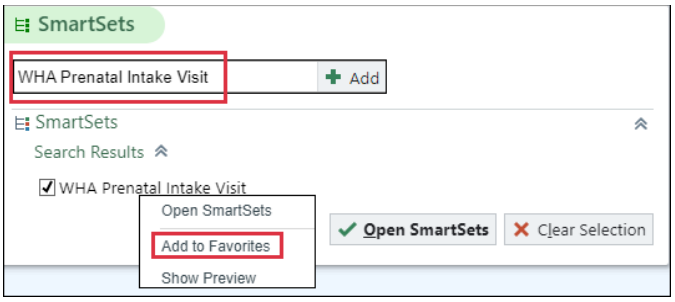
    a. This should be completed with the patient privately, so you may need to ask any guests present to excuse themselves until it is completed.
    b. Fill out the **Well Being Assessment** as thoroughly as possible then click the **Submit** button located at the bottom of the questionnaire.
29. Confirm that the patient has watched the **prenatal education video.** If not strongly encourage them to watch this prior to their next provider visit.
30. Review prenatal intake talking points.
31. Confirm they are scheduled for their 8 week OB PRIM appointment with a provider, discuss that patient should come prepared to talk further about genetic screening by this appointment. Also confirm that their 12 week OB workup, D&V ultrasound, and lab visit are scheduled. This can be confirmed in the **Appointment Desk**.
32. Click **Appt** button under the **Epic Menu** or from your **Hyperspace bar**.



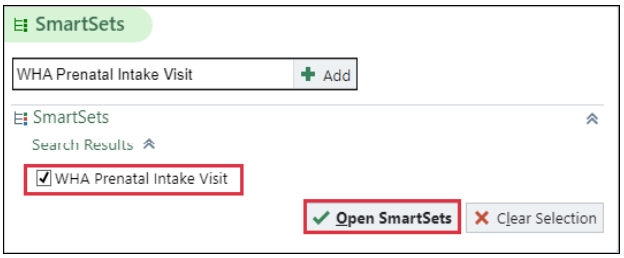
33. End the meeting with the patient.

This Visit Activity:

34. Go to the **This Visit** activity and scroll to your **SmartSet** section.
35. Search for the **WHA Prenatal Intake Visit** SmartSet, then add it to your favorites by right-clicking it and selecting **Add to Favorites**.



36. Open the SmartSet by checking the box next to the **WHA Prenatal Intake Visit** SmartSet then click the **Open SmartSet** button.



a. In each section there are small triangles that will expand the lists.

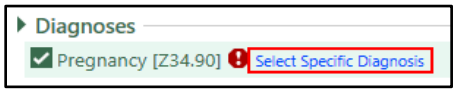


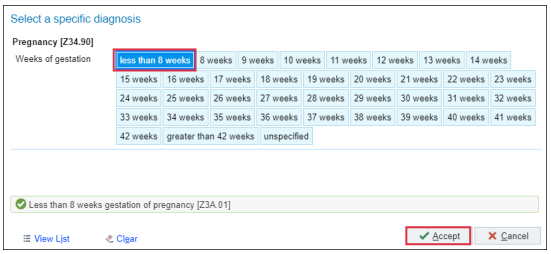
37. The chief complaint, pregnancy diagnosis and level of service will be pre-selected in the smart set. You will, however, need to select the prenatal intake note template unless you have created a speed button.



38. In the **Diagnoses** section the **Pregnancy** diagnosis is already selected. Clicking the blue hyperlink, **Select Specific Diagnosis**, will allow you to pick the gestational age.

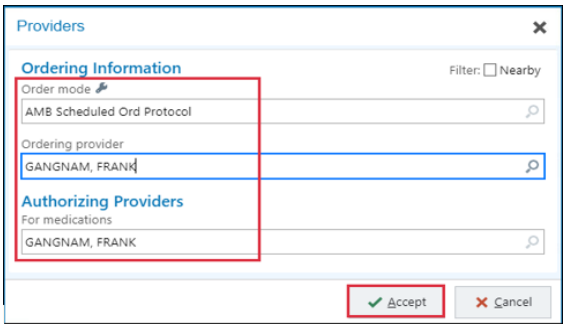
a. The pregnancy diagnosis is the only diagnosis that will be added at the intake regardless of patient history.



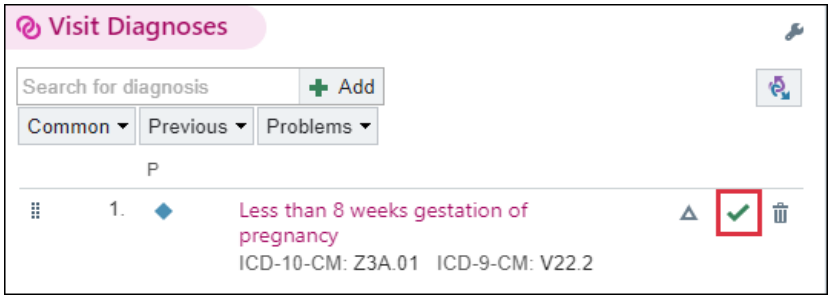


39. Use the **Referrals** section to place referral to Referral for Provider Consult as applicable.
40. Once you've selected all of the necessary components, click the **Sign**

***Note:** Make sure you sign this under the name of the provider doing the OB Workup if you have placed a referral order. **



41. In the **Visit Diagnosis** section click the **green plus**  to add the pregnancy diagnosis to the **Problem List**. Once clicked, it will change into a green check mark.

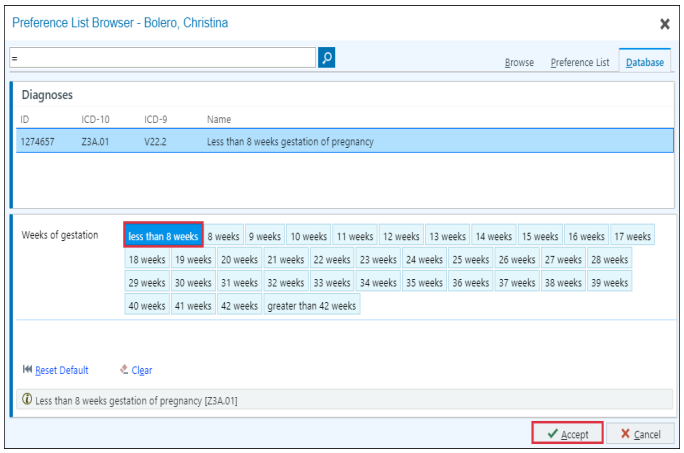


42. Once added to the Problem List, the diagnosis needs to be changed so that it is generic and not listed under a certain gestation and will be applicable for the whole pregnancy.

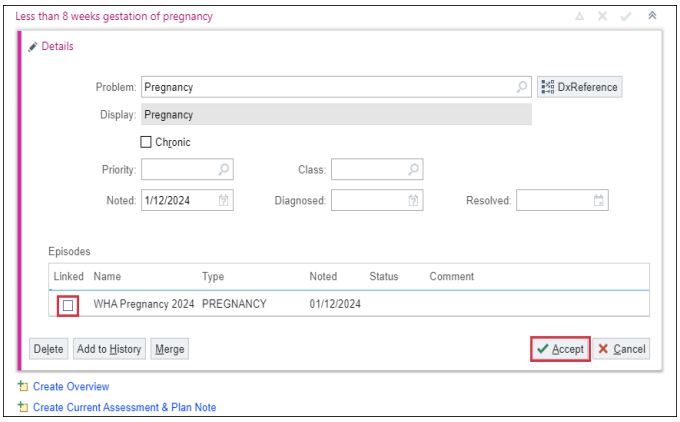
a. To do this, click the small triangle in the row of the problem you need to change.



b. Unselect the gestation.
c. Click **Accept**.

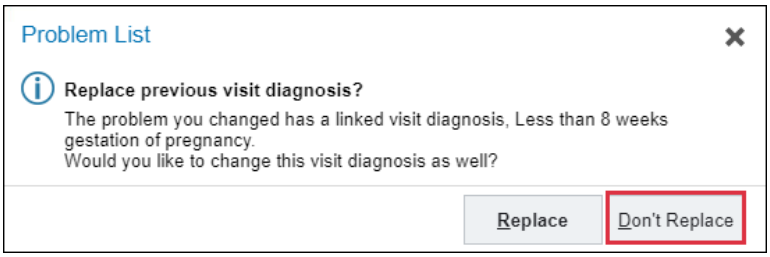


43. **Details** section will automatically open.
44. Under the **Episodes** section, click the box next to **WHA Pregnancy** to link it with the problem, then click **Accept**.



45. Epic will ask if you want to **Replace previous visit diagnosis**.

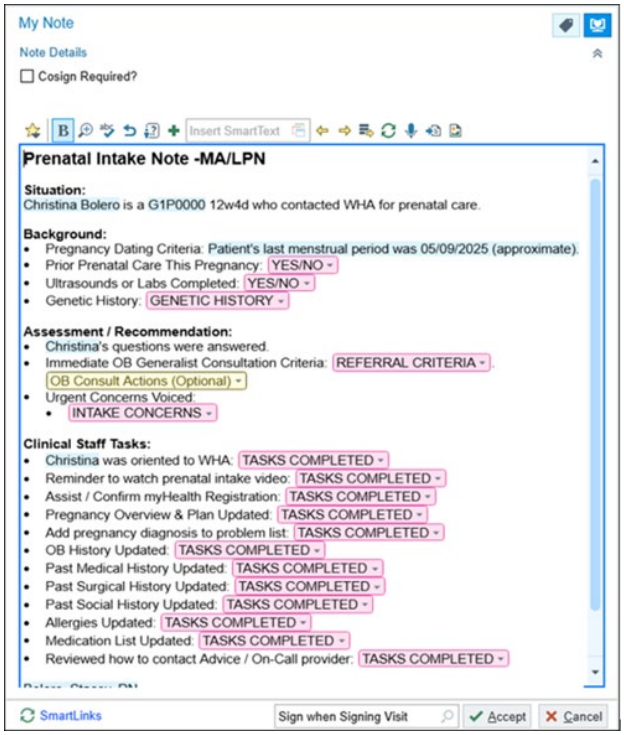
46. Click **Don't Replace**.



47. Click the **chevrons** to open the Pregnancy problem details.

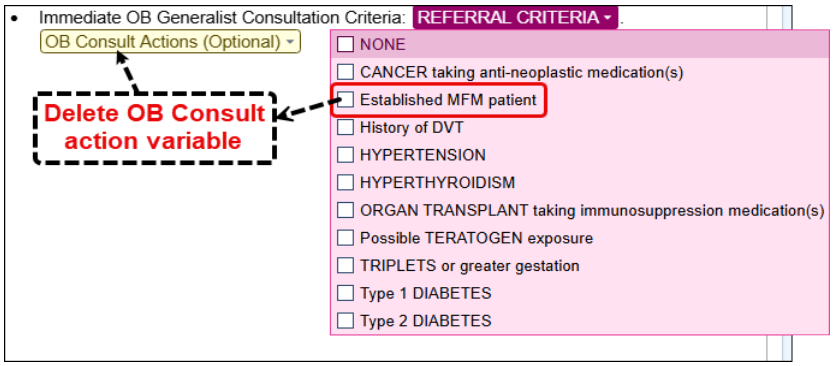


48. When signing the SmartSet, a note template should have been added to the **Progress Note**. F2 through the note to complete the variables and wild cards in the note.



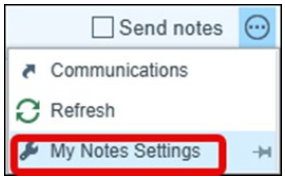
a. In the Immediate **OB Generalist Consultation** section, there is now a drop-down option labeled **Established MFM patient**. Please make sure to select this option if the appointment notes indicate Est MFM patient.

b. Delete the **OB Consult actions** variable as seen below, if they are an established MFM patient as no follow – up is needed.

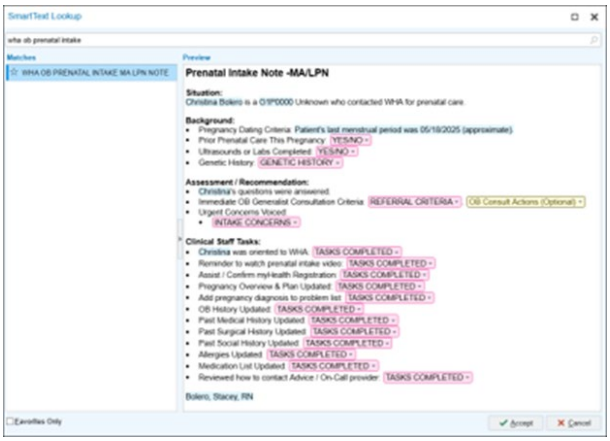


c. This note template can be saved as a button so that it can be added ahead of time and pre-charted in.

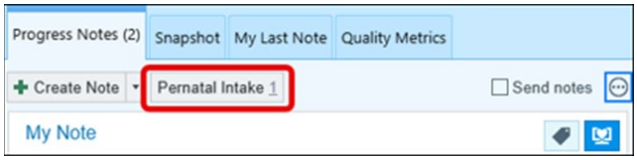
- Click the three ellipses  in the top right corner of the template and select **My Notes Settings.**



- In the **Speed Buttons** section, click **Create New**  .
- Add a **Caption** [Prenatal Intake, in the **SmartText** box, type **WHA OB PRENATAL INTAKE MA LPN NOTE**
- Press **Enter**
- Select the correct note template and click **Accept.** then click **Accept** again.

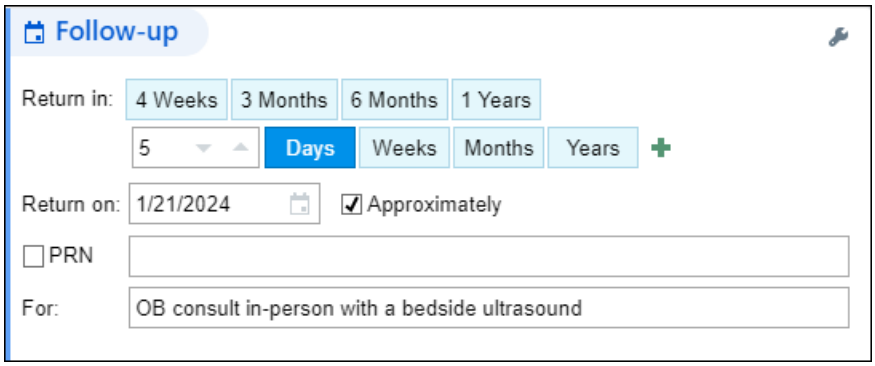


- Now, it's a speed button available for pre-charting.

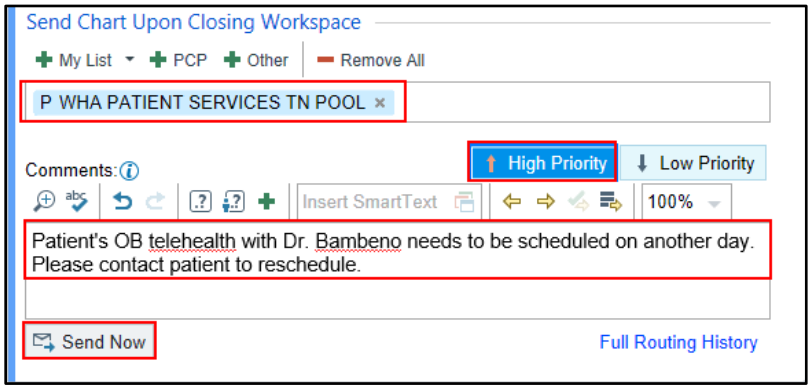


Wrapping up the encounter:

49. The SmartSet and Pregnancy Checklist should have populated all pertinent patient instructions in the **Patient Instructions** section.
50. All patients should be scheduled for an 8-10 week **OB PRIM CONSULT BSUS** or **OB PRIM CONSULT TELEHEALTH** [based on site and clinician] visit with a provider and a 12 week OB Workup with an ultrasound prior and a lab appointment after.
    a. Add the scheduled visit date in the **Follow Up** section, so patients know when to return. It will also show on their **After Visit Summary**.



    b. If something appears to be scheduled incorrectly, send an urgent staff message to the patient services pool of the patient's home site to have it fixed.



51. If a patient reports/request one of the below offer an OB triage visit with a provider. If the patient desires a visit connect with a patient services member to assist with scheduling. If the patient declines proceed with your intake as usual. There will be a section in the note template where you can document a declination or desire to schedule. Click here to access the OB Triage Referral Workflow. *Return to OB Triage*
    a. Nausea/vomiting
    b. Urinary symptoms
    c. Early conformation of pregnancy
    d. Hx of SAB/ectopic
    e. Medication safety questions
    f. Desires smoking cessation
    g. Positive COVID test
52. If a patient reports one of the below complete your intake and then open a telephone encounter. Document the concern/symptom and route as urgent to the patient's home site advice pool. After this has been done attempt a warm transfer the patient to the advice backline at the patient's home site. If advice is unavailable let the patient know that a nurse will be calling them back. There will be a section in the note template to document all steps taken.
    a. Shortness of breath/chest pain
    b. Bleeding/pelvic pain
    c. Dizziness or fainting
    d. Suicidal ideation
53. If a patient reports the below, they meet criteria for an urgent OB Generalist Consult. Click here to access the Referral for Provider Consult. *Return to Provider Consult*
    a. Diabetes Type 1/2
    b. History of DVT
    c. Hypertension
    d. Hyperthyroidism
    e. Triplets or > gestation
    f. Cancer currently on antineoplastic medication(s)
    g. Organ transplant currently on immunosuppressant medication
    h. Teratogenic medication exposure
    i. Established MFM patient

- As a reminder this is already a selectable option within the "referral criteria" section in the note template.
- This option should only be selected for patients who have already initiated care with MFM during their *current* pregnancy.
- This option *should not* be used for patients who were seen by MFM in a previous pregnancy.

54. Place referral to Referral for Provider Consult through the smart set. Include in the referral notes why the patient is being referred.
    a. The remainder of this workflow will be managed by our LPNs on the advice team.
    b. There will be a section in your note template to document steps taken.
55. Click **Sign Encounter.**

Prenatal Intake Talking Points

1. Genetics testing decision
    a. Remind patient to come to the 8-10 week visit prepared to discuss their options for genetic screening. Ideally, they should have checked insurance benefits for any testing they are interested in prior to this appointment.
    b. Notify patient that genetic screening information will go to their MyHealth account.
        - This information will not include specifics on billing codes so you will need to direct to the "patient resources" section on the WHA website
2. Provider types; male/female practice
    a. Remind patient that if they need to be seen and their provider is not available they will see one of their partners which could include:
        i. OB/GYN physician (male/female)
        ii. Certified nurse midwife
        iii. Physician's assistant
        iv. Nurse Practitioner
3. How to contact the office & on call provider
    a. For urgent after hours needs call the office and the clinician on call will be paged.
    b. We ask that you reach out during normal business for any non-urgent needs, either via phone or your MyHealth account. Allow a 72 hour response time if the patient has reached out via MyHealth.
4. Bleeding precautions
    a. Bleeding in early pregnancy may or may not be normal. Please call the office if you have any bleeding.
5. Does the patient have any questions about the content of the video?
6. MyHealth set up (if time allows)

Return to the Prenatal Intake Checklist

FAQ/ Q&A for Early Detailed Anatomy Orders

What if the patient asks about qualifying factors?

You can explain that there are a variety of clinical considerations providers use to determine whether a patient qualifies for a detailed anatomy scan. Unfortunately, we don't have access to all of this clinical information. Their OB Prime provider will review any relevant risk factors and determine the most appropriate type of scan. For specific questions, the provider is the best resource.

Why would a detailed scan be recommended?

A detailed anatomy scan allows for a more in-depth evaluation of the baby's development, which may be necessary when certain risk factors are present. The OB Prime provider uses multiple criteria to make this decision and will discuss it with the patient if applicable. Any further questions about qualification or medical necessity should be directed back to the OB Prime provider.

Why is it labeled as "genetic screening" if I opted out of genetic testing?

Although you may have opted out of genetic testing, the detailed anatomy scan includes the measurement of nuchal translucency- this is a standard part of the scan and is required per clinical and bilingual guidelines. Due to the way these scans are coded for insurance purposes, they may be grouped under the category of genetic screening, even if no additional genetic testing is being performed.

How can I find out if my insurance will cover this?

Patients should be referred to our Financial Counseling team, who can help review insurance benefits, determine coverage, and estimate any out-of-pocket costs. This ensures they receive accurate and personalized information.

Chromosome Screening

NIPT

a. One part test that screens for Down syndrome, trisomy 18/13 and fetal sex chromosome abnormalities.
b. Blood is drawn at 12 week OB workup although can be drawn as early at 10 weeks 0 days.
c. Recommended screening test for all patients.

Carrier Testing

1) Cystic Fibrosis

a. 1/25 people are carriers.
b. Most people are silent carriers meaning the disease does not affect them and they often have no family history.
c. Screening should be offered to all patients regardless of age or what pregnancy this is.
d. If a patient's screening is positive they will speak with a genetic counselor about testing for their partner.
e. The patient does not need to repeat this test if they have done it before.

2) Spinal Muscular Atrophy

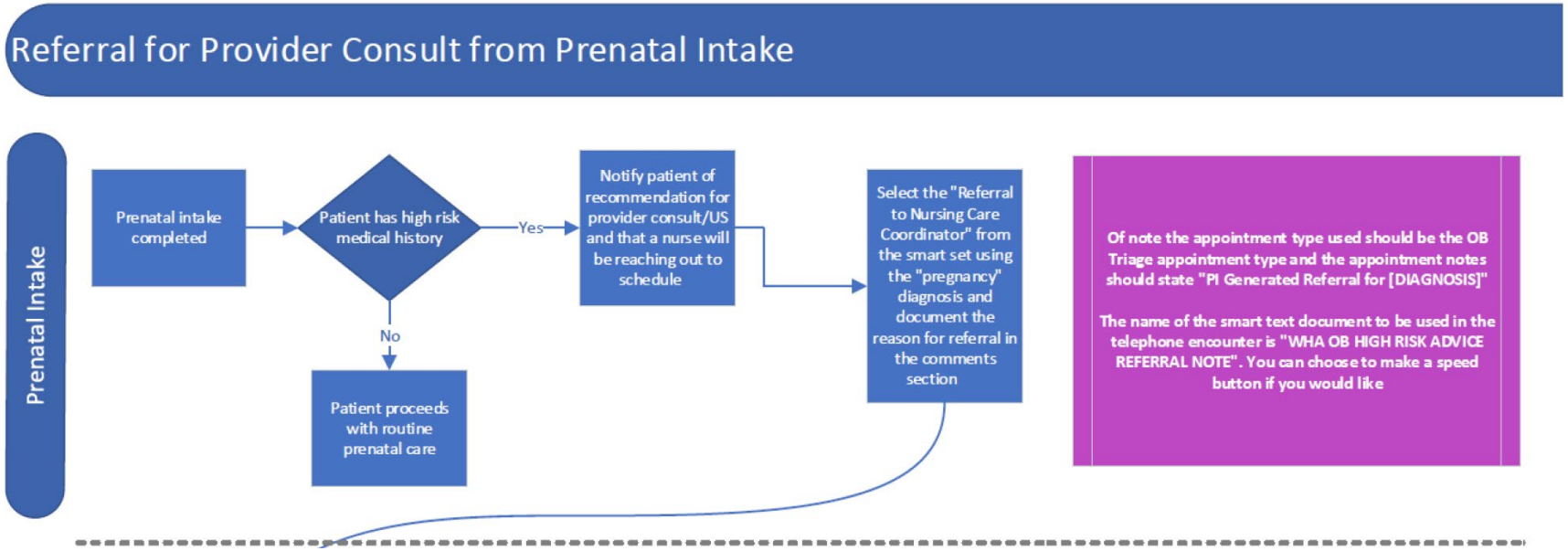
a. 1/40 people are carriers
b. Most people are silent carriers meaning the disease does not affect them and they often have no family history.
c. Screening should be offered to all patients regardless of age or what pregnancy this is.
d. If a patient's screening is positive they will speak with a genetic counselor about testing for their partner.
e. The patient does not need to repeat this test if they have done it before.

IVF

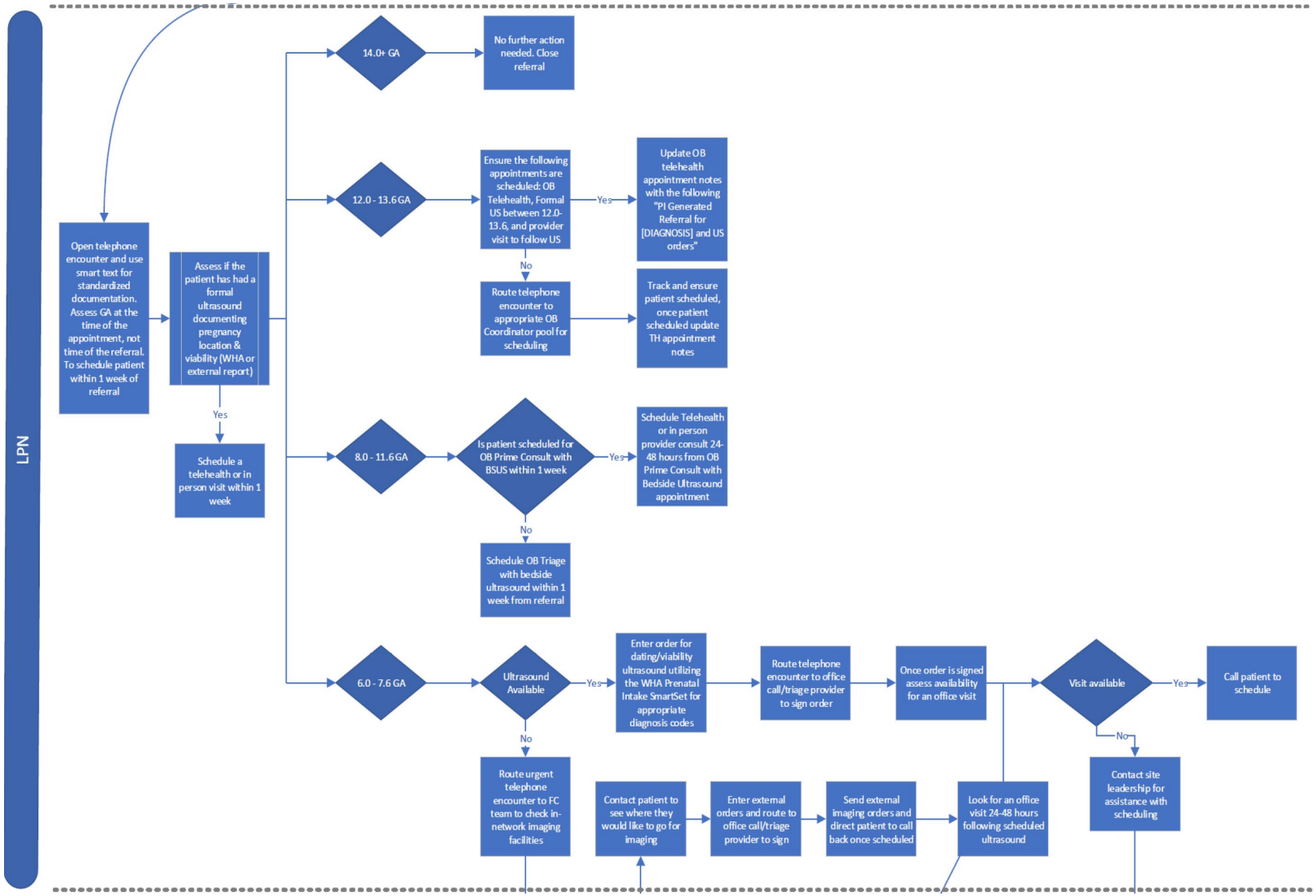
1. What we need to know:

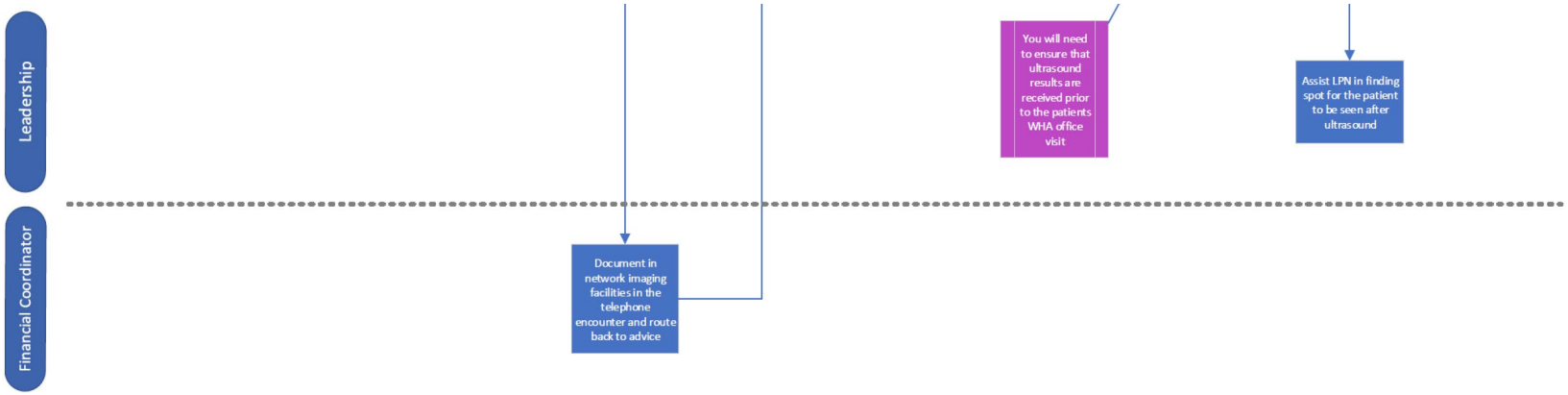
    a. Did the patient have PGT-A (preimplantation genetic testing-aneuploidy?
        i. If a patient wants to know how PGT-A compares to additional screening that is available to them they should be referred for a genetic consult.
        ii. PGT-A is screening and has a 98% detection rate.
    b. Did they have carrier testing?
    c. Did they use a donor egg?
        i. If a donor egg was used then obtain the donor's age if possible.

Referral for Provider Consult

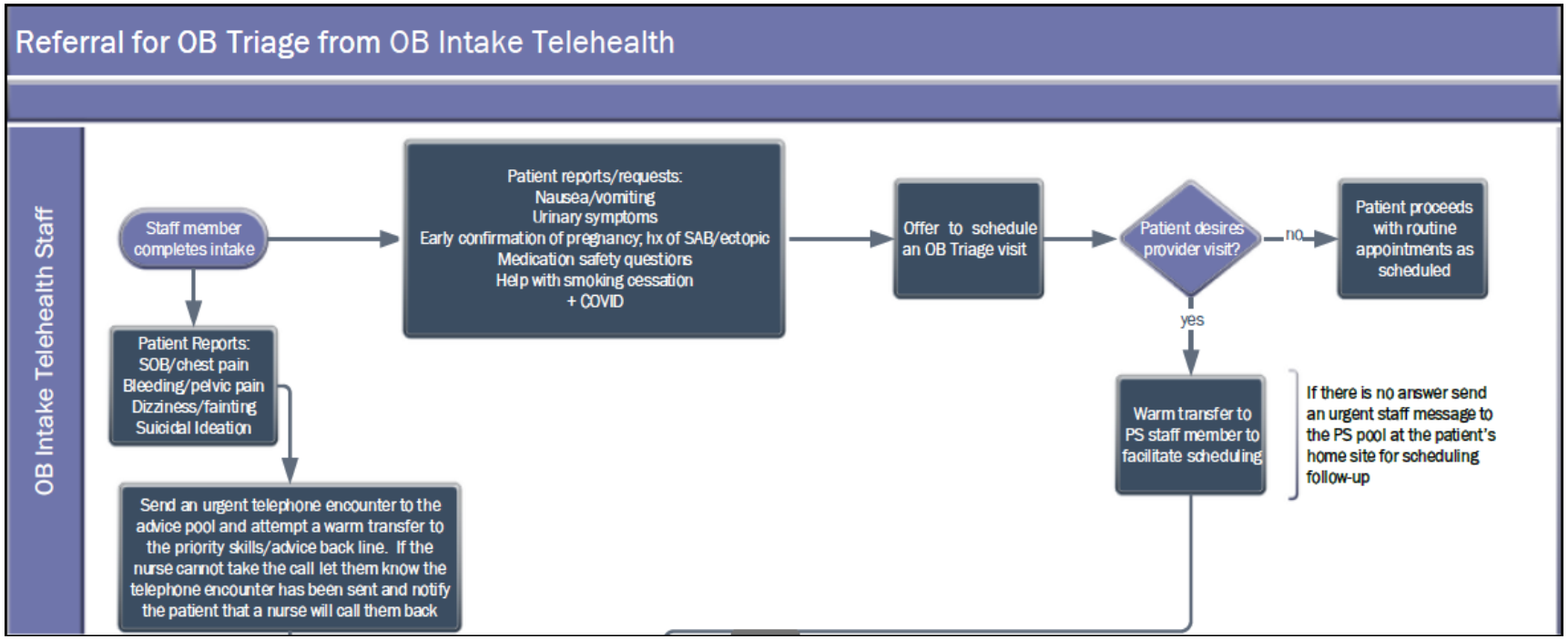


Return to Provider Consult hyperlink

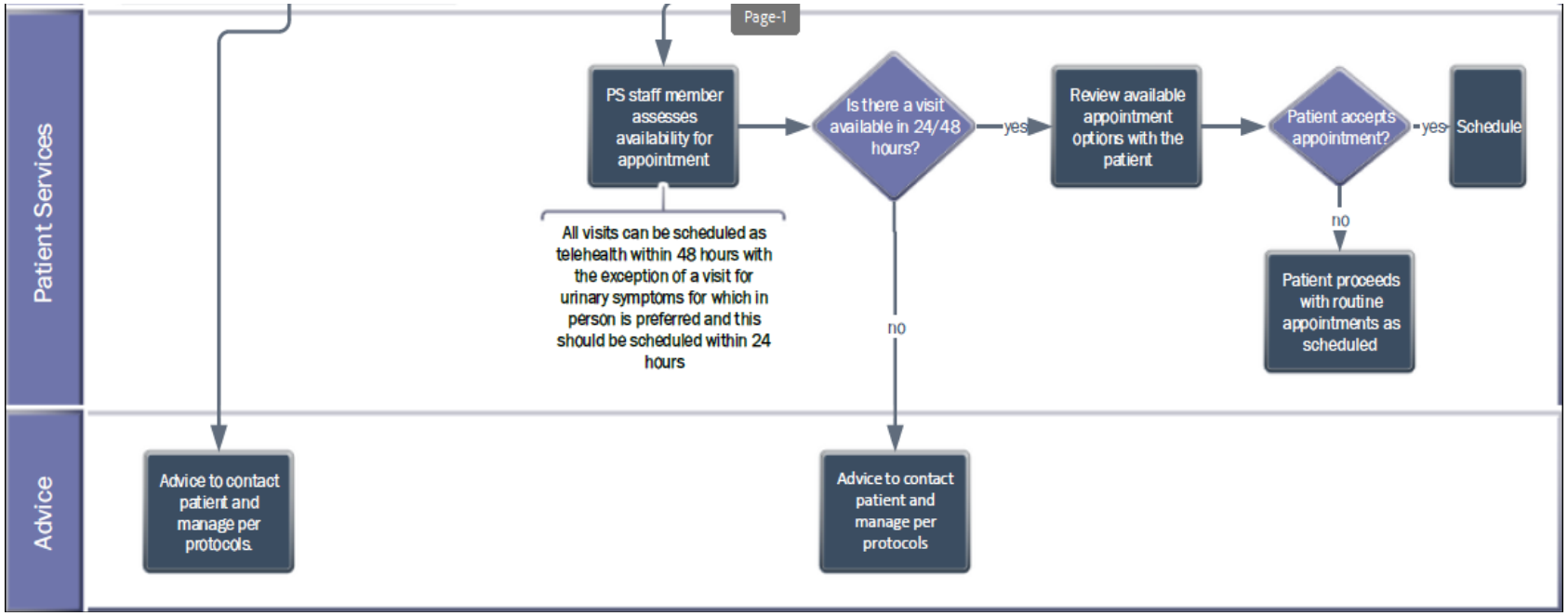

LPN
Open telephone encounter and use smart text for standardized documentation. Assess GA at the time of the appointment, not time of the referral. To schedule patient within 1 week of referral
Assess if the patient has had a formal ultrasound documenting pregnancy location & viability (WHA or external report)
Yes
Schedule a telehealth or in person visit within 1 week
14.0+ GA
No further action needed. Close referral
12.0 - 13.6 GA
Ensure the following appointments are scheduled: OB Telehealth, Formal US between 12.0-13.6, and provider visit to follow US
Yes
Update OB telehealth appointment notes with the following "PI Generated Referral for [DIAGNOSIS] and US orders"
No
Route telephone encounter to appropriate OB Coordinator pool for scheduling
Track and ensure patient scheduled, once patient scheduled update TH appointment notes
8.0 - 11.6 GA
Is patient scheduled for OB Prime Consult with BSUS within 1 week
Yes
Schedule Telehealth or in person provider consult 24-48 hours from OB Prime Consult with Bedside Ultrasound appointment
No
Schedule OB Triage with bedside ultrasound within 1 week from referral
6.0 - 7.6 GA
Ultrasound Available
Yes
Enter order for dating/viability ultrasound utilizing the WHA Prenatal Intake SmartSet for appropriate diagnosis codes
Route telephone encounter to office call/triage provider to sign order
Once order is signed assess availability for an office visit
Visit available
Yes
Call patient to schedule
No
Contact site leadership for assistance with scheduling
No
Route urgent telephone encounter to FC team to check in-network imaging facilities
Contact patient to see where they would like to go for imaging
Enter external orders and route to office call/triage provider to sign
Send external imaging orders and direct patient to call back once scheduled
Look for an office visit 24-48 hours following scheduled ultrasound


Leadership
Financial Coordinator
Document in network imaging facilities in the telephone encounter and route back to advice
You will need to ensure that ultrasound results are received prior to the patients WHA office visit
Assist LPN in finding spot for the patient to be seen after ultrasound

OB Triage Referral



Return to OB Triage Hyperlink


Page-1
Patient Services
Advice
PS staff member assesses availability for appointment
Is there a visit available in 24/48 hours?
yes
Review available appointment options with the patient
Patient accepts appointment?
yes
Schedule
no
Patient proceeds with routine appointments as scheduled
All visits can be scheduled as telehealth within 48 hours with the exception of a visit for urinary symptoms for which in person is preferred and this should be scheduled within 24 hours
no
Advice to contact patient and manage per protocols.
Advice to contact patient and manage per protocols